SDG Goal 3	Good health and well-being
SDG Target 3.3	By 2030, end the epidemics of AIDS, tuberculosis, malaria and neglected tropical diseases and combat hepatitis, water-borne diseases and other communicable diseases
SDG Indicator 3.3.4	Hepatitis B incidence per 100,000 population
Time series	Hepatitis B incidence

1. General information on the time series
   - Date of national metadata: May 7, 2026
   - National data: https://sdg-indicators.de/3-3-4
   - Definition: The time series shows the number of reported hepatitis B cases per 100,000 inhabitants.
   - Disaggregation: Not available.
2. Comparability with the UN metadata
   - Date of UN metadata: July 2026
   - UN metadata: https://unstats.un.org/sdgs/metadata/files/Metadata-03-03-04.pdf
   - The time series is not compliant with the UN metadata, but provides additional information.
3. Data description
   - The Infection Protection Act (Infektionsschutzgesetz, IfSG), which came into force in January 2001, regulates the statutory notification of certain communicable diseases in Germany. Notifications are generally submitted by medical doctors and laboratories and are mandatory. Nevertheless, underreporting may occur, as not all diagnosed notifiable diseases are fully recorded within the reporting system. The Hepatitis B cases reported by the Robert Koch Institute (RKI) are based on reports in accordance with Section 6 (1) (1) (e) of the IfSG.

     Until 2015, the case definitions for hepatitis B primarily focused on acute cases of disease, where clinical symptoms were present in conjunction with corresponding laboratory evidence. With the revision of the case definitions in 2015, surveillance was more strongly aligned with standardized diagnostic criteria, thereby improving the comparability of reported data.

     In 2017, various amendments were introduced to the IfSG as well as to notification and reporting procedures. These changes were primarily aimed at further developing surveillance structures and improving the completeness, quality, and comparability of data collected for public health reporting purposes.

     Further revisions of the case definitions were implemented in 2019. These served in particular to update diagnostic standards and to further harmonize the epidemiological assessment of notifiable diseases.

The population data are taken from the population updates of the Federal Statistical Office. For the calculation of population data, the retroactively calculated average population is used. For 2010, this is based on the 2011 census as well as migration, birth, and death statistics; for the years 2012 to 2021, the calculation is based on the 2022 census.

4. Access to data source
   - Online database SurvStat@RKI 2.0: https://survstat.rki.de/default.aspx
   - Infectious Disease Epidemiology – Annual Report: https://www.rki.de/EN/News/Publications/Epidemiology-Annual-Report/infectious-disease-epidemiology-annual-report-node.html
   - Average population – GENESIS online 12411-0041: https://www-genesis.destatis.de/genesis//online?operation=table&code=12411-0041&bypass=true&levelindex=1&levelid=1639396599054#abreadcrumb
   - Population data based on Census 2022 (only available in German): https://www.destatis.de/DE/Themen/Gesellschaft-Umwelt/Bevoelkerung/Bevoelkerungsstand/_inhalt.html#sprg233540

5. Metadata on source data
   - Online database SurvStat@RKI 2.0 - Content: https://survstat.rki.de/Content/Instruction/Content.aspx
   - Quality Report – Microcensus (only available in German): https://www.destatis.de/DE/Methoden/Qualitaet/Qualitaetsberichte/Bevoelkerung/einfuehrung.html

6. Timeliness and frequency
   - Timeliness: t + 3 weeks after data received
   - Frequency: Annual

7. Calculation method
   - Unit of measurement: Per 100,000 inhabitants
   - Calculation: Not applicable.